

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Jun 30, 2026

with Stella Lee, MD at Brigham and Women's Faulkner Otolaryngology Clinic



Notes from Care Team



Progress Notes by Stella Lee, MD at 6/30/2026 10:00 AM



BRIGHAM AND
WOMEN'S HOSPITAL



HARVARD
MEDICAL SCHOOL

Otolaryngology-Head and Neck Surgery

45 Francis Street, Boston MA 02115

Name: Daniel May

MRN: 10829169

Chief Complaint: No chief complaint on file.

Referred by: Unknown, Unknown, MD

PCP: Clark, Joshua W, NP

6/30/2026

History of Present Illness:

Thank you very much for referring Daniel S May to me for evaluation. As you know, this is a 36 y.o. male who presents with a history of two septal surgeries presents for evaluation of chronic sinonasal symptoms, including persistent nasal fullness, thick sticky mucus production, and ear and throat involvement ongoing for 2 years.

History of Present Illness

A 36-year-old male

Persistent symptoms include a sensation of fullness and stickiness in the sinuses, ears, and throat. Thick, sticky mucus is produced daily, requiring frequent nose blowing (100-200 times per day), coughing up phlegm, and occasional vomiting. Nasal discharge is difficult to expel and sometimes appears as large, cohesive plugs. Symptoms have worsened recently, with increased discomfort and swelling, particularly on the left side. He reports a

clicking sensation in both ears and the anterior nose, intermittent hearing loss, and a history of ear infections. An ear tube was placed but did not provide relief. No constant ear drainage is present. He reports that these symptoms have significantly impacted daily functioning, including the ability to work and participate in social activities.

He reports exposure to mold and dust in his home environment, particularly during basement cleaning and property preparation, and has not yet moved out of this environment. He has not undergone allergy testing. Use of xylitol spray and various over-the-counter biofilm disruptors, including Manuka honey, provided limited or transient benefit. He uses a tongue scraper, which produces white residue, and has developed new aphthous ulcers on his gums.

He describes gastrointestinal symptoms, including abnormal stools with mucus, stomach discomfort, and sensations of movement or spasm. He perceives a connection between his sinus and gastrointestinal symptoms but has not seen a gastroenterologist.

PAST SURGICAL HISTORY:

Septal surgery: 2014

Septal surgery: Date not specified

SOCIAL HISTORY

He works in an office setting at an early education child care business, handling real estate.

FAMILY HISTORY

He reports no family history of ciliary problems.

Radiology Findings: see below results.

CURRENT MEDICATIONS:

His medications were updated and reviewed in the Epic chart.

ALLERGIES:

has no known allergies.

PAST MEDICAL, SURGICAL, FAMILY & SOCIAL HISTORY:

Below data was updated and reviewed.

Past Medical History: has a past medical history of Trauma to eye.

Past Surgical History: has a past surgical history that includes Sinus surgery (Bilateral).

Family History: family history is not on file.

Social History: reports that he has never smoked. He has never used smokeless tobacco. He reports that he does not currently use alcohol. He reports that he does not currently use drugs.

REVIEW OF SYSTEMS:

Denies fever, chills, nausea, vomiting, vision changes, dysphagia, odynophagia, SOB, chest pain, rash, weight loss.

PHYSICAL EXAM:

Physical Exam

Ears: Right tympanic membrane appears normal. No drainage observed from the right ear.

Nose: Copious, thick clear drainage along the floor of the left inferior meatus. Middle meatus appears clear. Sphenoethmoid recess is clear. Olfactory cleft and superior turbinate appear normal. No polyps or purulence observed.

Oral Cavity: Tongue appears healthy. Base of tongue is normal. Aphthous ulcers present on the gums.

Throat: **Pooling of frothy mucus in the piriform sinuses. Vocal cords demonstrate normal mobility. Cobblestoning observed in the hypopharynx with frothy mucus throughout.**

Neck: Small pustules observed on the neck consistent with acne.

Skin: Small pustules present on the neck.

Specialty Assessment: Nasal endoscopy revealed copious, thick clear drainage along the left inferior meatus, clear middle meatus, clear sphenoethmoid recess, and thick clear mucus in the nasopharynx. No polyps or pus observed. Flexible laryngoscopy demonstrated pooling of frothy mucus in the piriform sinuses, normal vocal cord mobility, and cobblestoning in the hypopharynx.

General Appearance: well appearing, in no apparent distress

Facial Function: intact and symmetric

Eyes: No periorbital edema

Extraocular Movements: intact

Vision: grossly intact

Ears: RIGHT: External ear healthy. Otoscopic exam: External canal is without erythema. Tympanic membrane visualized and with no fluid in the middle ear

LEFT: External ear healthy. Otoscopic exam: External canal is without erythema. Tympanic membrane visualized and with no fluid in the middle ear

External nose: no deformity

Anterior Rhinoscopy: caudal septum no lesions, inferior turbinates anteriorly without lesions

Oral Cavity and Oropharynx: no erythema, no lesions, palate is symmetric, tongue is midline and fully mobile, no discolored drainage from the nasopharynx

Respiratory: Breathing comfortably, with no stertor or stridor

Skin: No vascular lesions or rashes on the face or the neck

Neurologic: CN3-7 grossly intact

PROCEDURE: Sinonasal Endoscopy

Based on the patient's history and physical exam, I recommended proceeding with bilateral nasal endoscopy to evaluate the olfactory clefts and sinus outflow tracts not visualized on anterior rhinoscopy. Verbal consent obtained. Endoscope attached to video tower, equipment calibrated.

Anterior and interior nasal cavity: There is no crusting or lesions in the nasal vault or on the septum.

Septum: clear

Right side: Inferior turbinate clear

Middle turbinate normal

Middle meatus: no evidence of infection

Superior meatus: no evidence of infection to the extent visualized

Sphenoethmoid recess: Clear

Left side: Inferior turbinate clear, copious, thick clear drainage along the left inferior meatus

Middle turbinate normal
Middle meatus: no evidence of infection
Superior meatus: no evidence of infection to the extent
visualized
Sphenoethmoid recess: Clear

Nasopharynx: No masses, Fossa of Rosenmuller clear, eustachian tube
orifices are unremarkable

RESULTS: IMAGES & DATA REVIEW

I personally reviewed the images and reports with the patient and concur with the radiology report. My interpretation is below.

CT Sinus:

Results

The following results were reviewed by myself.

Imaging

- Sinus scan: 08/2025, Obstructed outflow tracts bilaterally, more pronounced on the left. Septum is slightly deviated to the left.
- Sinus scan: 04/17/2026, Sinuses are clear. Maxillary sinuses are not filled with mucus, polyps, or biofilm. Frontal sinuses appear normal.

Daniel S May

Assessment & Plan

1. Other chronic sinusitis

Persistent thick nasal and pharyngeal mucus with sinus fullness for 2 years. Imaging from 08/2025 showed severe sinus blockages, but 04/17/2026 imaging appeared normal.

- Discussed environmental exposures and inflammation as likely contributors. Reviewed prior imaging and explained findings. Provided education on proper nasal rinse technique, avoiding forceful irrigation and nose blowing to protect cilia. Advised against self-cleaning moldy environments and recommended professional remediation. Instructed to collect biofilm samples for analysis.
- Ordered sinus swab for culture. Ordered updated CT scan of the sinuses.
- Initiate nasal rinses twice daily using distilled water, premixed saline packets, and budesonide liquid.
- Refer to allergy/immunology for comprehensive evaluation and possible allergy shots.

2. Non-seasonal allergic rhinitis due to other allergic trigger

3. Other irritable bowel syndrome

Chronic gastrointestinal symptoms with mucus in stools and abnormal motility.

- Discussed possible connection between sinus and GI symptoms including LPRD.

- Refer to gastroenterology for evaluation including parasitic workup.

Thank you for involving me in the patient's care. Please do not hesitate to contact me should you have any questions or concerns.

[Stella Lee, MD](#)

Sinus and Endoscopic Skull Base Surgery

Associate Surgeon

Otolaryngology-Head and Neck Surgery

Center for Surgery and Public Health

Brigham and Women's Hospital

Harvard Medical School

Office: 617-525-6500

brighamandwomens.org

BRIGHAM HEALTH



BRIGHAM AND
WOMEN'S HOSPITAL



HARVARD MEDICAL SCHOOL
TEACHING HOSPITAL

CC:

Clark, Joshua W, NP

Unknown, Unknown, MD

I personally spent a total of 33 minutes on care for this patient on the date of the encounter. This includes face-to-face time during the visit as well as non face-to-face time spent on chart review, radiographic imaging, documentation, testing, and care coordination and excluding the time spent in performance of the procedure.

I obtained verbal consent from the patient or their proxy to record this visit

for purposes of producing a draft of the encounter documentation.

MyChart® licensed from Epic Systems Corporation© 1999 - 2026